



Policy

SCGOPCHG Use of Restrictive Practices in a Non-Authorised Setting

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Hospital-Wide	All Staff

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Introduction

Sir Charles Gairdner Osborne Park Health Care Group (SCGOPHCG) is committed to ensuring the safety of patients, staff, and the public. It is the responsibility of SCGOPHCG to ensure safe practice in the management of Acute Severe Behavioural Disturbances (ASBD/ABD), including, where possible to prevent, reduce and eliminate the use of restrictive practices.

SCGOPHCG recognises that there are circumstances where restrictive practices may need to be applied to minimise imminent risk to the patient, visitors, and staff.

Restraints must only ever be used as an intervention of least restrictive and last resort.

Restrictive practices are performed/commenced by appropriately trained staff to prevent individuals from harming themselves endangering others or to ensure that essential medical treatment can be provided.

Risk Statement

Non-compliance with this policy may:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☒
Breach SCGOPHCG Mission & Values	☒	Staff Safety	☒
Other:	☐		

Definitions

Restrictive Practice	Any practice or intervention that has the effect of restricting the rights and freedoms or movement of individuals or groups. Restrictive practices can be defined as either detention, mechanical restraint, physical restraint, and/or environmental restraint
Restraint	The restriction of an individual's freedom of movement by physical and mechanical means.
Mechanical Restraint	The restraint of a person by the application of a device (which may include any mechanical material, appliance, or equipment) to prevent, restrict, or subdue a person's movement. Mechanical Restraint does not include the use of devices for therapeutic or non-behavioural purpose. For example, allied health devices for safety/therapeutic purposes or cot sides for safety.
2 point limb restraint	Restraint involving two limbs
4 point limb restraint	Restraint involving four limbs
6 point limb restraint	Restraint involving four limbs and both elbows
Physical Restraint	The application by staff members of 'hands-on' immobilisation or the physical restriction of a person to prevent the person from harming themselves or endangering others, to ensure the provision of essential medical treatment. Physical restraint does not include the use of a physical support in a way to guide or redirect a person away from potential harm/injury, consistent with what could reasonably be considered the exercise of care towards a person.

Environmental Restraint	Any action which restricts a person's free access to all parts of their environment, including items or activities.
De-escalation	The use of verbal and non-verbal communication skills aimed at defusing anger and averting aggression.
Acute Severe Behavioural Disturbance (ASBD)	Can be used interchangeably with Acute Behavioural Disturbance (ABD) or Behaviours of Concern (BoC) and puts the patient or others at risk of harm and may include threatening or aggressive behaviour, extreme agitation or distress, self-harm which could cause injury or death.
Detention	The act of keeping back, restraining, or withholding, either accidentally or by design, a person or thing.
Senior Clinician	Is the senior most person available be it a Medical Officer or Senior Registered Nurse (SRN), <u>trained in restraint authorisation/application</u>

AIN / NURSE / SECURITY SPECIAL is **not** considered a form of restraint. Placing a person on special observation is an escalation of care with the aim of providing an increased level of support, communication and early escalation that could not otherwise be achieved within normal nursing ratios. Please refer to [SCGOPHCG Clinical Companion and Patient Special Management Policy](#) for specific roles of the special.

Ethical considerations

Restraints not only cause acute distress during the experience but also may exert long - term effects including self-stigmatisation, traumatic memories and even symptoms of post-traumatic stress disorder (PTSD)

The use of restraints in patients with a history of trauma or abuse may lead to re-traumatisation. A trauma informed approach should be used by all clinicians to understand and respond to the impact of trauma on the patient, their physical physiological and interpersonal responses within health care. The key principles of trauma informed care (TIC) are safety; trustworthiness and transparency; peer support; collaboration and mutual support; empowerment (voice and choice); and cultural and historical sensitivity.

Policy Principles

If, at any time, staff feel threatened or unsafe in anyway, immediately call a CODE BLACK (dial 55) to ensure prompt attendance by clinical and security staff.

The following criteria need to be met prior to engaging in the use of restrictive practices:

- All reasonable lower risk and less restrictive options have been attempted or considered to de-escalate or manage the behaviour of the patient and/or the imminent or actual risk they pose.
- The patient lacks the capacity to make informed decisions about their healthcare needs at the time, or the patient is of uncertain capacity and is experiencing an acute severe behavioural disturbance and requires an urgent assessment of their capacity and/or treatment of the cause of the behavioural disturbance.
- The patient is at significant imminent risk of causing serious harm to self or others.

Restraints must **NEVER** be used:

- For the convenience of staff or to overcome inadequate supervision, setting and/or equipment.
- Used to punish or negatively reinforce behaviours of concern.

- If at any time you deem a situation too dangerous and/or there is a perceived or actual weapon in use, stand back, activate code black or duress alarm, keep yourself and if possible, others safe.
- If staff available do not have appropriate information, instruction, or training.

Doctrine of Necessity is a common law doctrine developed on a case-by-case basis and may authorise restraint and detention where:

- The restraint was done to avoid an imminent peril to life or serious injury.
- The person carrying out the restraint believed the individual was placing him or herself or others in a situation of imminent peril.
- The restraint was proportionate to the potential harm that could have occurred.

Post Restraint Principles

Any interference with a patient's rights, privacy or dignity must be the minimum necessary to protect the safety of all, particularly when the restraint occurs in public areas and shared facilities.

It remains the responsibility of the primary nurse to deliver all personal care, document and perform required restraint and physiological observations, and escalate concerns as required. All persons involved in restraint will have the opportunity to request a hot debrief with the SRN in attendance or can be referred to NMHS wellbeing for staff and eReferral to Clinical Psychology for patients.

All patients who are restrained must have the below completed:

- Behavioural Observation Form (BOF)
- Code Black Record – **if applicable**
- AIN/PCA Record and/or Security Handover Record – if applicable
- Mechanical Restraint and Observation Chart
- Restraint Tracking Tool
- BERT referral

Physical Restraint

Physical restraint can be applied if a plausible threat, attempt or actual episode of self-harm, injury to another person or persistent destruction of property has occurred. Any staff member involved in physically restraining an individual **MUST** have completed specific training in restraint techniques and have a sound knowledge of relevant legislations. In situations where restraint(s) is deemed necessary or required, including a Code Black or planned intervention, the most senior clinician available may verbally authorise the use of restraints. As far as practicable preventative interventions including de-escalation and/or conflict resolution should be attempted prior to physical restraint application.

- The purpose of physical restraint is to safely immobilise the individual concerned.
- It may precede or be necessary prior to pharmacological management or the application of mechanical restraint.
- A coordinated, team approach, with clearly defined roles prior to any action taken.
- Equipment, bed space, traffic minimisation should be considered, if safe to do so.
- Consideration of the implications and risks of being involved in applying restraint should be explored.
- If staff have not been trained in the application of restraint, and deemed physical intervention necessary, they are to activate a Code Black immediately, ensure their safety, stand back and await Security and SRN assistance.

If physical restraint is required, the force used must be considered 'reasonable'. Reasonable force must be:

- Absolutely necessary – no other alternative.
- Proportionate – the degree, duration and the nature of the force used in relation to the harm, to be avoided.
- Reasonable in accordance with Australian Law

Response Team

Senior Registered Nurse (SRN) Clinical Incident Lead

- Obtain handover on arrival, including medical/physical considerations.
- Risk assess, plan and coordinate team response to incident, including the safety of all parties involved.
- Communicate with the person being restrained in a continual attempt to de-escalate the situation and inform them of what is occurring.
- Leads the team through the situation, including all forms of restraint.
- Continual assessment of the persons safety, including A-E assessment and ensuring documentation of observations.
- Ensure completion of all relevant restraint documentation including the integrated progress note, restraint tracking tool, and Mental Health Act 2014 forms present if applicable.
- Debrief post incident with staff, visitors and with patient when appropriate.

Senior Medical Lead –Emergency Department for all code blacks and inpatient areas when requested by clinical incident lead (Home Team Registrar or CAT Registrar)

- Risk assess in collaboration with the SRN team leader to coordinate team response to incident.
- Assess patients' capacity to make decisions.
- Maintain airway, observe vital signs and monitor for any sign of respiratory compromise during restraint and feedback observations to team.
- Support and protect the head and neck during restraint if required and it is safe to do so.
- Communicate with the person being restrained in a continual attempt to de-escalate the situation and inform them of what is occurring.
- Continually monitor interventions of other team members.
- Post incident review of patient, short- and long-term management plan including documentation of management plans in the integrated progress notes.
- Debrief post incident with staff, visitors and with patient when appropriate.

Clinical team members

- Ensure good team communication use clear and concise statements, along with closed loop communication; alert SRN Team Leader and / Senior Medical Lead to new information as it becomes available. Ask for help/clarification if required.
- Medication preparation and obtaining equipment to maintain patient and staff safety, including protecting the privacy and dignity of the patient.
- Ensure accurate documentation of observations, medications given and time of restraint application and release.

- Post-incident care and observations are carried out and documented with any concerns escalated to the SRN Lead/Medical Lead or medical emergency team called.
- Attend debriefing session lead by team lead.

Security

- Security Officers to activate audio and visual recording devices on their way to incident.
- Obtain handover on arrival, including medical/physical considerations that may affect the safety of the patient during the application of restraint.
- Security officers are guided by the senior most clinician leading the incident.
- Security officers have all been trained in the application of restraint and aware of the key principles in physical restraint positioning.
- Security to complete electronic sentry report post every incident.
- Attend debriefing session lead by team lead.

Safety Principles to be observed for all Positions.

A patient may be physically restrained in several positions depending on the level of aggression, physiological concerns, and individual risk assessment. Each position has its own risks of harm to patient and staff (Refer to [Table 1](#)).

Supine restraint is the preferred physical restraint position, and all physical restraint should return to this position as soon as it is safe to do so.

Below are the principles that are applicable for all positions:

- 1 During restraint, under no circumstances should DIRECT pressure be applied to the neck, trachea, thorax, abdomen, back or pelvic area.**
- 2 The nose and mouth should never be occlude/covered unless medically approved e.g., oxygen mask.**
- 3 The airway must be continually assessed, and the head protected if necessary.**
- 4 There should be no deliberate use of techniques whereby a person is allowed to fall, unsupported, to the floor. When moving a patient from a standing position to prone or supine on the floor, the patient must be supported during the descent and protected from any impact.**

Table 1 - Restraint Positioning and Associated Risks

Restraint Position	Risks	Risk reduction strategies
Prone (Face down)	Sudden unexpected death. May restrict the ability to breathe lessening the supply of oxygen to meet the body's needs. Should only be used as an intervention of last resort when no other method/ position of restraint is feasible.	Continual patient monitoring (Airway, Breathing, Circulation) and communication. Only staff trained in the application of restraint should carry out the intervention. No pressure placed on the head, neck, or back. Patient not to be placed in prone maximal restraint position (hogtied) Limited to the minimum amount of time necessary to change position, preferably kneel or standing. NOT GREATER THAN 3 MINUTES a senior clinician must be actively involved in such cases.

Supine (Face up)	Choking or inhalation of vomit and aspiration. Risk of asphyxia, fatal cardiac arrhythmia, or respiratory distress	Keep patients head or bed elevated to reduce aspiration risk.
Seated	Patient leaning forward may increase risk of harm or death during prolonged restraint. Risk further increased if a higher BMI. When patient's arms are wrapped around the front of the body and held from behind - increases risk of asphyxia.	Patient to be seated upright. Avoid hyperflexion of the spine Do not wrap patient's arms around the front of their body in the 'basket hold' position.



If concerns regarding **airway** are raised, restraint should **STOP immediately**.

If a patient physically deteriorates or reports that they are unable to breath, an A-E physiological assessment is to occur and identified issues must be addressed.

Activate MET immediately.

Risk associated with Physical Restraint Use

There are no absolute safe restraint interventions. How 'hazardous' a restraint position is may be individualised, depending on patient characteristics, duration of restraint, the forcefulness of the hold and a range of other factors including the levels of stress and the extent of aggressive attitudes projected by all concerned.

Depending on the environment, the specific health risks of the patient may or may not be known. All approved restraint techniques must consider the possibility that underlying disease(s) may render an individual more susceptible to adverse effects and possibly death.

Physical restraint may increase the risk of death, falls, serious injury and increased length of hospital stay.

There is a risk of death from obstructing airways during physical restraint, but harm can also occur after the event.

Staff must never cause deliberate pain to a patient to force compliance.

Please refer to:

- Appendix 1 – Patient risk factors for physical restraint
- Appendix 2 – Adverse outcomes during physical restraint application

When to Remove Physical Restraint

A collaborative discussion has occurred between SRN, shift coordinator (SCO) and medical officer to cease physical restraint when the below has been considered:

- Chemical and potentially mechanical restraint has been applied and effective.
- It is safe to do so, recommence communication with patient taking care not to re trigger the situation.
- The confrontation has ended/subsided.
- Clinical deterioration.

- Behaviours of concern are no longer an imminent risk.
- Discharge/transfer.

Pharmacological Management

Pharmacological management involves, the use of medication for the purpose of influencing a person's behaviour, other than medication prescribed for the treatment of, or to enable treatment of, a diagnosed mental disorder, a physical illness, or a physical condition.

Anticipating the need for pharmacological management early, may assist in reducing the need for physical and mechanical restraints. Pharmacological management should be aimed at achieving maximum efficacy through regular timed dosing, without the necessity to resort to PRN.

In situations where there are difficulties achieving this stability, advice should be sought from the treating Medical team or Charlies Afterhours Team (CAT) to review and further inform continued prescribing rationale.

The aim of pharmacological management is to achieve a state of calm, sufficient to minimise the risk posed to themselves and others, this could vary dependant on situation. The patient's usual ability to respond to communication should ideally, not be compromised because of pharmacological restraint.

Type of Pharmacological Management

Oral medication – is a reasonable first line option in the cooperative patient and is the preferred route as consent is implied when willingly ingested. Oral medication may be as effective as a parenteral form (usually Intramuscular-IM or Intravascular- IV), for the agitated but nonviolent patient willing to cooperate with treatment.

Rapid Sedation – is the use of medication by parenteral route (usually IM or IV) if oral medications is not possible or appropriate and urgent sedation with medication is needed. Should only be used on a patient who is highly aroused, agitated, overactive, aggressive, or making serious threats or gestures towards others, themselves, or is being destructive to their surroundings, when other therapeutic interventions have failed to deescalate the behaviour.

The use of medication to manage acutely disturbed behaviour is a very short-term strategy designed solely to reduce immediate risk; this is distinct from treating any underlying mental illness.

The degree of sedation required must be balanced against potential side effects.

All staff involved in prescribing and/or administering medications and monitoring the effects of rapid sedation should operate within their scope of practice and be aware of the indications, contraindications and side-effects of the medications used in context of the patients' comorbidities, age, current medications and potential associated toxidromes.

In time - critical situations where a written prescription would delay administration and have an impact on patient outcomes, including S8/S4R medications a senior member of the treating medical team or CAT Registrar can provide a verbal order.

The Behavioural Observation Form (BOF) must be commenced if not already in use.

Please refer to the relevant pathway for guidelines to sedation in ASBD:

[Identification, Escalation and Management of Behavioural Disturbances \(BD\)](#)

[Cognitive Impairment Management \(Delirium and/or Dementia\)](#)

Observations post Pharmacological Management

Level/type of sedation	Observations Required
Oral	Physiological - when safe to do so or as clinically indicated. Commence BOF
IM	Physiological - when safe to do so, at least 15 minutes; then 1 hour post administration; then as clinically indicated. Commence BOF
IV	At 5 minutes, then every 15 minutes for 1 hour; 30 minutely for 2 hours: then as clinically indicated. Continuous pulse oximetry during sedation and for at least 1 hour post administration. Activate MET call if concerned. Commence BOF
Any sedation that leads to a reduced level of consciousness / unrousable	Same as IV, plus and when safe to do so ECG Bladder scan Sedation score Position head of bed at 30*

Mechanical Restraint

Mechanical restraints are the most restrictive form of restraint that deprives a person of their ability to mobilise. They must only be used when no other alternative option is available. In situations where restraint(s) is deemed necessary or required, including a Code Black or planned intervention, the most senior clinician available may verbally authorise the use of restraints.

Reasons for the utilisation of mechanical restraint come under the following reasons:

- To provide lifesaving care and treatment and to prevent the removal of essential medical equipment such as invasive lines, tubes, medications, and dressings.
- To minimise the mobilisation of a patient where that mobilisation may cause imminent serious harm to the patient.
- For patients whose behaviour poses an imminent risk to others and all other levels of intervention have been deemed ineffective or unsuccessful in managing that risk.

Risks of mechanical restraint use

- Injury and/or death have been associated with incorrect application of restraint devices. Even when straps are correctly applied vigilance is required.
- Deaths have been caused solely by mechanical restraints, especially in patients who were not continuously observed. Causes of death have included strangulation and chest compression.
- Where prolonged mechanical restraint is used, the risk of forced immobilisation and potential for pressure injuries, deep vein thrombosis (DVT) and nerve damage must be considered as part of the management plan.

- All staff involved in the application and removal of mechanical restraint devices must be trained to apply them correctly to prevent adverse outcomes to staff and patients i.e., when restraints are removed from the patient, they are also removed from the bed/chair.

Mechanical Restraint Authorisation

Mechanical restraint authorisation should be a collaborative discussion between senior Medical and Nursing staff, with all alternative strategies exhausted and the least restrictive option utilised. Medical and Nursing staff should acknowledge the use of restrictive practices in the inpatient notes and reauthorise as outlined below.

If restraint use has been verbally authorised, the treating Medical team, Senior Registered Nurse (SRN) or After Hours Medical Officer **must** review the patient within 30 minutes.

The SRN commencing the mechanical restraint is to complete all relevant documentation including Mechanical Restraint Authorisation and Observation Chart MR410/858 in its entirety, enter the initial restraint onto the Restraint Tracking Tool and summarise the events in the integrated progress note.

The therapeutic use of restraint is to protect medical devices being removed and the use of occupational therapy devices i.e. positioning in chairs, splints for limbs etc. ***An SRN 3+ must authorise the first use of a susan table, mittens and cot rails and sides (for restriction), stroke chairs or tilt high back chairs (prevention of FALLS only)***, every 24 hours thereafter the ward Shift Coordinator may authorise.

The MR410/858 is to be reauthorised as per below:

- Use of Susan tables fitted to appropriate chair and therapeutic mittens every 24 hours.
- Cot rails and sides (for restriction of movement only), stroke chairs or tilt high back chairs (prevention of FALLS only) every 24 hours
- Limb restraints, lap belts, restrictive mittens every 6 hours

The SRN and / or Shift Coordinator will be responsible for updating the authorisation of therapeutic devices every 24 hours and the Shift Coordinator / Registered Nurse caring for the patient will be responsible for updating the Restraint Tracking Tool for each application and removal of all other restrictive practices.

Provision of care

Patients in mechanical restraints are reliant on staff for their basic care needs. Care will include:

- Remove / repositioning of restraint. Temporary release of the restraint for the purpose of caring for a patient's needs (for example, toileting, feeding, and range of motion) a minimum of 2 hourly if ongoing use is indicated.
- Regular mobilisation as safe to do so to prevent deconditioning.
- Ensure call bell can be activated and is in reach of patient if safe to do so.

Types of Mechanical Restraint

The following mechanical restraints are in use at SCGOPHCG include:

Type of restraint Device description	Risk of Device	Level of monitoring
Limb Restraints Pelican blue Limb restraints with Velcro  Note: SCGOPH staff will only be trained to use this limb restraint. For any alternate limb restraints, please arrange additional training for your area.	- Oedema and ischemia of patient's wrists and ankles - Pressure injury/skin tear - DVT - Aspiration especially when used in conjunction with chemical sedation. 2- point arm restraint risk ability for patient to flip themselves over the back of the bed, compromising shoulder injury NEVER restrain lower limbs ONLY	1:1 Special RN/AIN whilst restrictive practice in use - Physiological observations 15 minutes after application, hourly for 4 hours then as per ORC chart. - Neurovascular observations 15 minutes after application, hourly thereafter. - Reposition patient 2 hourly. Minimum of 1 hourly review of patient and reassessment of ongoing requirement for restraint by RN. SRN authorisation 6 hrs. Medical awareness and documentation per 24hrs
Pelican Soft Lap Belts  	Patient slipping off chair and potential strangulation/chest compression/neck injury.	30 minute visual observation. - Reposition patient 2 hourly. - Confirm if voided in last hour. - Ensure that the patient has not slipped down the chair. Minimum of 1 hourly review of patient and reassessment of ongoing requirement for restraint by RN. SRN authorisation 6 hrs. Medical awareness and documentation per 24hrs

Use of Restrictive Practices in a Non-Authorised Setting

Mittens 	• Oedema and cyanosis of patients wrists • Increased agitation due to frustration	30 minute visual observation. • Neurovascular observations 15 minutes after application, hourly thereafter. • Reposition patient 2 hourly. • Confirm if voided in last hour. Minimum of 1 hourly review of patient and reassessment of ongoing requirement for restraint by RN. 1st Application authorised by SRN 3+, every 24 hrs thereafter Shift Coordinator. Medical awareness and documentation per 24hrs
Susan Chairs with tables fitted on their arms. 	• Increased risk of adverse outcome if device fitted incorrectly. • Injury to the chest against table • Increased agitation • Falls • Position asphyxia from patient slumping in the chair	Hourly visual observation. • Reposition patient 2 hourly. • Confirm if voided in last hour • Ensure that the patient has not slipped down the chair or body parts are not pressing against T-bar Minimum of hourly review of patient and continued need for restraint by allocated RN. 1st Application authorised by SRN 3+, every 24 hrs thereafter Shift Coordinator. Medical awareness and documentation per 24hrs
Stroke and Tilt Back Chairs <u>** FALLS ONLY**</u>	• Falls	• Hourly visual observation to ensure patient seated in chair correctly • Reposition patient 2 hourly or as required. 1st Application authorised by SRN 3+, every 24 hrs thereafter Shift Coordinator. Medical awareness and documentation per 24hrs <u>**FALLS ONLY**</u>
Bed Rails and Cot Sides <u>**when used to restrict movement**</u>	• Ensure all limbs are free from entwinement. • Falls	Hourly visual observation. • Reposition patient 2 hourly or as required. 1st Application authorised by SRN 3+, every 24 hrs thereafter Shift Coordinator. Medical awareness and documentation

		per 24hrs <u>**ONLY when used to restrict movement**</u>
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Principles of limb restraint applications

Consider which limbs are to be restrained; aim for least restrictive application:

1- point restraint: applied by security/SRN.

- **DO NOT** restrain a single arm unless **unilateral weakness** or medically indicated as this may result in the person placing themselves in a compromising position with the potential for position asphyxia or removal of own restraints.

2 - Point restraints: applied by security/SRN.

- Can be used on both arms to prevent removal of life saving medical devices. Or one arm and opposite leg. Consider indwelling devices, wounds etc. when initially placing restraint.
- If using blue pelican limb restraints, use of crepe bandage/velband underneath to protect skin integrity and comfort for patient.
- **NEVER** apply 2-point restraint to just lower limbs

4 or 6 - point restraints: applied by security/SRN.

- To be used **only** in situations where the patient is too violent to contain in any less restrictive means.
- Arm and body positioning to be considered for comfort and pressure injury management, ie., lateral lying and one arm up one arm down. Consider patients ability to tamper/remove restraint device and potential weaponisation of same.
- 4 - 6 point restraint change limb positions 2 hourly.

Limb Restraint Release and Removal

Release

If there is an ongoing need to restraint, release one limb restraint at a time and reapply before releasing another limb restraint. Consider resources that may be required in the event of a re-escalation. When releasing a four-six-point restraint, do so slowly -usually one point at a time – to reduce the patient's risk of harm to self or others.

Once removed, assess each limb for sign if injury/pressure.

Removal

If during release/removal of restraint the patient's behaviour escalates, reapply the last effective level of restraint. Notify the Shift Coordinator, security or call a code black as appropriate.

Order of removal of restraint:

1. Remove elbow restraints (if 6-point restraint)
2. **Allow a period of observation** to review the risk of behaviours of concern and clinical needs before removing the selected limb restraint. If during release / removal of restraint the patients behaviour escalates, reapply the last effective level

of restraint. Notify the SRN, Shift Coordinator, security or call a code black as appropriate.

3. Remove lower limb restraints and repeat step 2.
4. Remove upper limb restraint, ideally non-dominant limb first, and repeat step 2
5. Remove remaining upper limb restraint (dominant) and repeat step 2.

Do not remove both restraints from same side, ie left upper and left lower limb restraint, as this may allow the patient to flip themselves off the bed or leave patient restrained by one limb only unobserved, unless the patient has unilateral weakness. Ensure time of restraint released is recorded on the Mechanical Restraint Authorisation Chart MR 410/858 and in the integrated progress note.

Special Considerations

Pregnancy

Pregnant (>20wks) patients should **NOT** lay flat, care should be taken to position the pregnant patient in the left lateral position to avoid the risk of aortic compression by the uterus.

Mental Health Act

Consider MHA requirements and consult Clinical Liaison Psychiatry Nursing for inpatients during hours, on call Psychiatrist after hours and in the ED Psychiatry Liaison Nurse after hours.

Older Adults

When managing agitation in older adults with dementia or delirium, refer to [Cognitive Impairment Management \(Delirium and/or Dementia\)](#)

Cleaning and decontamination of re-usable restraints

- All re-usable patient care equipment must be cleaned and decontaminated after each patient use. Please send to Hospital Equipment Store (HES) in heavy duty bag.

Compliance and Monitoring

Both the Department of Health and National Safety and Quality Healthcare Standard 5 require regular reporting on the rates of all forms of restrictive practice. Evidence to be provided include:

- Code Black and restrictive (all forms) practice use,
- De-escalation and prevention practices to reduce the incidents of restrictive practice,
- Capacity and consent,
- The use of supplementary support from security officers,
- Injuries resulting from restrictive practices,
- Notification of next of kin, guardian, or substitute decision maker,
- Multidisciplinary reviews conducted.

To assist capture this data a restraint tracking tool has been created along with a code black form.

Restraint Status Form

The restraint status form is used to monitor the rates of restrictive practice within SCGOPHCG.

- Initially be completed by the **SRN authorising the restraint for the first time**, including the co-occurring conditions, whether the patient is under the MHA 2014 and the notification of the next of kin.
- For **ongoing use** of therapeutic equipment such as a Susan Table, Mittens or the use of cot rails and sides to restrict movement can be authorised every 24 hours thereafter by the Shift Coordinator (SC).
- The SC or the Registered Nurse (RN) caring for the patient will be required to complete the Restraint Status Form every time a restraint is used during the authorised period and the time the restraint was removed.
- This should be recorded as soon as practical after initiating or removing restraint.

The RN is still required to complete the Mechanical Restraint Authorisation and Observation Chart MR410/858.

Pharmacological management is required to be entered in the below circumstances:

- During a code black, regardless of administration route
- If SRN or medical review has resulted in a one-off dose of an antipsychotic or sedative, regardless of administration route
- Is an IM or IV alternative to PO route that has been prescribed in the PRN section of the medication chart.

Exclusion for entering pharmacological management:

- Patient given a prescribed PRN PO medication of antipsychotic.
- Management of acute withdrawals (unless during a code black)
- Immediate post operative management in PACU (unless during a code black)
- Intensive care patients receiving ongoing behavioural management, unless a code black is called the additional medication should be entered onto the restraint status form
- Any medication prescribed for the treatment of, or to enable treatment of, a diagnosed mental disorder, a physical illness, or a physical condition.

This information will populate into a dashboard hourly and will include the number of code blacks called in the previous 24 hours.

Code Black Record

The Code Black Record will need to be completed post every code black, by the SRN in the response team and emailed to [SCGH, BERT](#) who will collate and enter the data into the dashboard.

Post-Restraint Support and Debriefing

Staff:

Being involved in or the witnessing of restraint can have lasting psychological impact on all involved. Timely and tailored support is essential in the reduction of negative outcomes of traumatic experiences.

Following any episode of restraint, all persons involved, including the patient, who witnessed/involved in the incident should be offered [Psychological First Aid](#) and counselling where appropriate.

Additional supports options include.

- A hot debriefing session may be asked for and delivered by the SRN 3+ immediately after an incident.
- Further clinical/operational debriefing or ward/group Psychological first aid sessions are a voluntary process. Staff should be informed of the purpose of the session, and sessions should be offered at a suitable time for staff to be able to attend if interested.
- To clarify support options for staff, leaders may benefit from contacting [NMHS Wellbeing](#)

Patient:

- Any incident involving the use of restraint, that results in the injury of a patient, must be reported via the Clinical Incident Management System (Datix CIMS) by Clinical Staff.
- An episode of restrictive practice can be traumatic. It is important to acknowledge the element of trauma in this and past experiences and take necessary steps to manage this trauma. Clinical Psychology, Consultation Liaison Psychiatry and Aboriginal Health Liaison Officers should be considered as a part of this process.
- Patients have a right to refuse to engage in a post restraint de-brief, but every effort should be made by staff to engage the patient. If a patient chooses not to engage in a post-restraint de-brief, this should be documented in the patients notes.

Multidisciplinary Resources

Clinical resources

- Behavioural Escalation Response Team (BERT) 0700-2130
- SCGH Security Team Leader
- Ward NM and or CNS, relevant line manager during hours.
- AH CNS 1400 – 0730 weekdays and all hours over the weekend and public holidays.
- Alcohol and Other Drug
- Aboriginal Health Liaison Officer

Mental Health

- Consultant Liaison Psychiatric Team: via eReferral
- Psychology: via eReferral
- Emergency Department PLN: Ext 76727 between the hours of 2130 - 0700

Work Health and Safety

[Health, Safety and Wellbeing](#)

Related Documents

Legislation

- [Mental Health Act 2014](#)
- [Mental Health Regulations 2015](#)
- [Criminal Code Compilation Act 1913](#)
- [Work Health and Safety Act 2020](#)
- [Guardianship and Administration Act 1990](#)
- [Privacy Act 1988](#)
- [The Aged Care Act 1997](#)

Authorising Policy and Standard/s:

- [Mental Health Policy Framework](#)
- [DOH Use of Restrictive Practices in Non-Authorised Healthcare Settings Policy](#)
- [WA Health Code of Conduct](#)

- [DOH WA Health Consent to Treatment Policy](#)
- [DOH Workplace Aggression and Violence Policy](#)
- [Clinical Handover Policy](#)
- [NMHS Security-Use of Force](#)
- [Australian Charter of Healthcare Rights](#)
- [Authorisation of restrictive practices resources](#)
- [Chemical restraint](#)

[National Consensus Statement: Essential elements for recognising and responding to deterioration in a person's mental state | Australian Commission on Safety and Quality in Health Care](#)

[ACSQHS Delirium Clinical Care Standard](#)

Standards, procedures, guidelines:

- [SCGH Mechanical Restraint Minimisation Nursing Practice Guideline](#)
- [Guideline Use of Detention and Restraint in Non-Authorised Healthcare Settings](#)
- [Guideline Psychotropic Medicines in Cognitive Disability or Impairment](#)
- [SCGOPHCG Detention of Patients and Discharge Against Medical Advice](#)
- [NMHS CIMS](#)
- [Falls Prevention | Australian Commission on Safety and Quality in Health Care](#)

Supporting documents

- [Regulated Restrictive Practices Guide \(ndiscommission.gov.au\)](#)
- [RPBG Safe Use and Management of Patient Restraints Clinical Practice Standard](#)
- [FSFHG Restraint in Non-Mental Health Areas Policy.](#)
- [Safe Use and management of Patient Restraints CPS for general areas only](#)
- [National Mental Health Commission-Report Rev \(unimelb.edu.au\)](#)

Authorisation & Version Control

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Policy Contact	Coordinator of Nursing – BERT				
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Standards Applicable					
Acknowledgment	RPBG Clinical Practice Standard: Minimisation and Safe Management of Restraint and Seclusion				
Aboriginal Health ISD Number:	2301				

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

Document Version Control			
Date	Version	Author	Notes
11/09/2025	0.16	Jacinda Smits	Update from Falls Guidelines, Restraint status form requirements, Medical and nursing authorisation, removal of Posey Jackets, addition of the IEM of BD policy link.

Appendix 1: Patient risk factors from Physical Restraint

Risk Factor	Rationale
Obesity, large abdominal girth	High risk of positional asphyxia if restrained in prone position as abdomen may restrict movement of diaphragm
Alcohol affected Prescribed or non-prescribed benzodiazepines	Respiratory depressant /increased risk of aspiration
Drug affected- stimulants	Prolonged struggle / physiological stress
Serious mental illness including psychosis/mania	Prolonged struggle / physiological stress Pharmacology: benzodiazepines, anti-psychotics with potential for increased QT interval and cardiac arrhythmias
Sickle cell trait	Exercise related collapse, local hypoxia creating sickled haemoglobin
Older adult	Risk to musculoskeletal system
Underlying cardiovascular or respiratory disorder	Exacerbation of symptoms
Pregnancy	Position this patient in the left lateral position to avoid the risk of aortic compression by the uterus
History of trauma	Exacerbate/re-experience trauma

Appendix 2: Adverse outcomes during Physical Restraint Application

Causes	Description
Positional asphyxia (Restraint asphyxia)	Obstruction of breathing as a result of restraint technique. It occurs when the position of a person's body interferes with their ability to breath.
Excited delirium/acute behavioural disturbance	Excited delirium syndrome is a constellation of signs and symptoms including, among other things, violent behaviour, agitation, delirium, imperviousness to pain, and superhuman strength. It is clinicopathological diagnosis that is ultimately made only after death, based on autopsy, toxicology, and historical context. Potential pathophysiologic explanations include hypoxia, hyperkalaemia, acidosis, autonomic dysfunction, and dopamine and catecholamine dysregulation, all of which could be impacted by struggle and restraint.
Exertion leading to acidosis	In patients who have been restrained, continued agitation, and struggle can lead to severe metabolic acidosis. The aetiology of this acidosis is unclear but probably involves the production of lactic acid from physical exertion compounded with sympathetic induced vasoconstriction
Stress-related cardiomyopathy	In restraint-related deaths, extreme physiological stress, and sudden exercise, e.g., violence and struggling, in an individual with genetic predisposing factors, may result in fatal hypertrophic cardiomyopathy
Catecholamine hyper stimulation	Patients with catecholamine hyper stimulation (such as blunt force trauma from agitation or other hyperactivity, physiological stress, hyperthermia, or illicit drug use) have a greater risk of sudden death during restraint